
CONSULT FOR AFRICA

Osteon Clinics

Audit Fieldwork Kit: Thursday 17 September

Internal, Consult for Africa audit team only. Not for the client.

Site: Osteon Clinics, Amuwo Odofin, Lagos

Prepared 15 September 2026



Posture, before anything else

We are here as a partner in the findings, not as an inspector. Dr Bola invited us in and he owns this business, so every member of his team will be watching how we treat him as much as how we treat them. Open with reassurance, listen far more than we talk, and never let a frontline person feel blamed for a gap that belongs to the system.

Standard opener for every conversation: "Thank you for the time. This is not about individuals, it is about how the place works and how to make it stronger. Nothing you tell me is attributed to you by name."

Three rules for the day. Write down what we see, not what we conclude, because the conclusions get built later and out of the building. Photograph everything we are allowed to photograph, with permission, and never with a patient in frame. Where a record does not exist, record that it does not exist and who told us so, because an absence is a finding and it needs the same evidence trail as a presence.

Nothing identifiable leaves the site. Case trace notes use our own case numbers, C1 to C10, and the patient identifiers stay on a single sheet that stays in the building or is destroyed before we leave.

Running order for the day

Time	What	Who leads
08:00	Arrive. Coffee with Dr Bola. Agree the day, confirm access, pick the ten cases	Debo
08:30	Observe: the clinic opening, or the theatre list being sent for	All
09:15	Full tour, front door to sterile room, with whoever runs the place day to day	All
10:00	Split. One of us on the ten-case trace in records. One of us on theatre and sterile services	Split
11:30	Physical counts: implant store, drugs, emergency trolley, asset spot check	One
12:00	Interviews begin, thirty to forty minutes each	Rotating
14:00	Observe a clinic consultation end to end, with patient consent, and time it	One
15:00	Front desk and money: watch an enquiry, a booking, a payment, a claim	One
16:00	Interview Dr Bola properly, and not in a corridor	Debo
17:00	Close with the point of contact. Name what is still outstanding, in writing, before we leave	Debo

If a theatre list is running, it takes priority over everything on this list. Watching one arthroplasty list from send-for to recovery is the most informative hour available to us and it cannot be reconstructed afterwards.

The tour: what to look at, room by room

Walk it in the order a patient walks it, because that is how the flow problems reveal themselves.

Approach, car park and entrance

- How does a patient on crutches or in a wheelchair get from a car to reception, and how far is it
- Signage from the road. Would a first-time patient find this
- Steps, ramps, door widths, thresholds
- What does the building say about the practice inside it, in the first five seconds



- Security, and what happens to a patient arriving after hours

Reception and waiting

- How many seats, and are they ever full
- Who is at the desk, how many phones ring at once, and what happens to the second caller
- Is there a written enquiry log, or is it WhatsApp on a personal phone
- Privacy at the desk. Can the waiting room hear a payment conversation
- Toilets, water, clock, visible price information
- Patient-facing material on display, and what it claims

Consulting rooms

- Number of rooms, and how many are actually used on a clinic day
- Examination couch, screening, hand hygiene point, sink
- Can the room next door hear the consultation
- Where does the doctor write, and into what
- Is there a light box or screen for imaging, and does the imaging arrive before the patient

Imaging

- Units present, make, age, and current working state
- Radiation warning signage, door interlocks, lead protection, staff badges on the wall
- Look for the NNRA licence and the last quality assurance report displayed or filed
- Where the images go, and how a report comes back
- Who is in the room during exposure

Theatre complex

- Number of theatres, and the state of the doors, walls, floor and ceiling
- Ventilation: is there laminar flow, where are the filters, when were they last changed, is there any record
- Scrub area, water supply, gowning and glove store
- Anaesthetic machine, monitor, suction, oxygen source, and the backup for each
- Table, lights, diathermy, tourniquet, C-arm in the room or brought in
- Doors: how many are there, do they close, and how often did they open during the case we watched
- Traffic during a case. Count the people in the room and the number of entries and exits
- Where is the implant opened, by whom, and where does the sticker go
- Emergency drugs in theatre, malignant hyperthermia provision, difficult airway kit
- What happens if the power goes now. Ask, then look for the evidence

Recovery

- Where it is, how many spaces, and who is in it
- Monitoring available, oxygen, suction, call bell
- Who watches a patient waking up, and what else are they doing at the same time
- Discharge criteria, written or in someone's head

Sterile services

This is the highest-consequence room in the building for an arthroplasty practice.



- On site or sent out. If sent out, to whom, and what comes back with the tray
- Dirty to clean flow. Is there one, or does everything happen on one bench
- Washer disinfectant or manual cleaning, and what protective equipment is worn
- Autoclave make, model, age, chamber size. Photograph the plate
- Service record and validation certificate, and when each was last done
- Cycle printout or log for the last three months. Take a photograph of a real week
- Daily air removal test and the chemical and biological indicator records
- Load traceability. Ask directly: if I give you a patient's name, can you tell me which sterilisation load their instruments came from
- Storage of sterile packs. Shelf, floor clearance, humidity, packaging integrity, date marking
- Any single use item being reprocessed. Ask, look, and note the answer verbatim

Implant and consumable store

- Locked or open, and who holds the key
- Systems present, sizes held, and whether stock is owned or on consignment
- Lot numbers visible and legible on the boxes
- Expiry dates on a sample of ten boxes
- Where loan kits sit when they arrive, and how their completeness is checked
- The stock record, and whether it is a book, a spreadsheet or nothing

Drugs and pharmacy

- What is held, where, and at what temperature
- Controlled drug cupboard, register and key holder
- Expiry check on a sample of twenty lines
- Who orders, from whom, and what a stock out has cost in the last year

Ward or inpatient beds

- Number of beds, spacing, and how many are ever occupied
- Call system, oxygen, suction at the bedside
- Who is here at two in the morning, and what they are qualified to do
- Where the notes live, and whether observations are actually charted

Physiotherapy

- Space, equipment, and who delivers the sessions
- Whether post-operative patients are seen on the ward or only as outpatients
- The protocol, if written

Records, office and money

- Where paper notes live, whether the cabinet locks, and who can open it
- The billing and accounting software on the screen. Ask to see today's takings
- Backups. Ask when a restore was last tested and accept the honest answer
- Claim files, and the oldest unpaid claim in the drawer

Plant, power, waste



- Generator make, size, hours, service record, fuel arrangement
- Changeover time when grid fails, and whether theatre has an uninterruptible supply
- Water source, storage, treatment
- Oxygen cylinder store and checking routine
- Clinical and sharps waste: where it accumulates, who collects it, and the consignment notes

The ten-case trace

Pick with Dr Bola on the morning: three primary arthroplasty, two revision, one infected joint, two major trauma, one that had a complication, one that never proceeded after being quoted. Spread across the last 18 months and across sites if possible.

For each case, work through this chain and mark every link present, absent, partial or not applicable. The value is in the chain, not in any single link, because a break anywhere in it tells us where the system loses hold of a patient.

Link	What we want to see	Note
1. First contact	The enquiry recorded anywhere, with a date	Is there a funnel at all
2. First consultation	Clinic note, legible, dated, with a plan	
3. Imaging	Images available, reported, filed with the record	
4. Decision to treat	Written, with the alternatives discussed	Supports the ethics-forward positioning
5. Quote	The amount quoted, what it covered, and who set it	
6. Deposit and payment	Date paid, amount, receipt, and how long after the quote	The single biggest funnel gap
7. Pre-operative assessment	Done, by whom, what was checked, and what would have stopped it	
8. Consent	Signed, dated, risks written, by the operating surgeon	
9. Theatre register	Case listed, start and finish times recorded	
10. Safety checklist	Completed and filed for this case	
11. Antibiotic prophylaxis	Given, and the time recorded relative to incision	Strongest infection lever there is
12. Implant record	Component lot or serial stickers in the notes, all of them	Recall traceability
13. Sterilisation record	The load this patient's instruments came from, identifiable	



Link	What we want to see	Note
14. Operation note	Written by the surgeon, same day, with implant detail	
15. Post-operative orders	Analgesia, thromboprophylaxis, mobilisation plan	
16. Nursing observations	Actually charted, not blank	
17. Discharge	Summary given, criteria met, follow-up date set	
18. Invoice	Raised, itemised, and matching the quote or explaining why not	
19. Collection	Received into the bank, date, amount, and any balance written off	Trace to the bank statement
20. Claim	If a payer was involved: submitted, approved, paid, or still sitting	
21. Follow up	Attendances at each interval, and the last time this patient was seen	
22. Outcome	Any score, any complication, any return to theatre, current status	The publishing question

Record for each case: total quoted, total collected, implant cost, days from quote to deposit, days from deposit to surgery, and length of stay. These twelve numbers across ten cases give us the first honest picture of case economics and funnel timing, and they are worth more than any estimate Dr Bola gives us from memory.

Counts and spot checks

Implant stock count. Physically count every implant box in the store, by system, size and lot. Compare to whatever record exists. Note anything expired, anything with a damaged package, and anything nobody can explain the presence of. Value it at the last purchase price.

Autoclave cycle records. Take one full week at random from the last three months. Count cycles run, cycles with a recorded chemical indicator result, cycles with a biological indicator, and days with a recorded air removal test. Photograph the log.

Emergency trolley. Contents against a written standard if one exists, and against the standard we brought if one does not. Check seal, check every expiry, check the defibrillator turns on and its pads are in date, check oxygen and suction work, check the last recorded check. Ask two staff members, separately, where it is and what is in it.

Drug expiry. Twenty lines at random, including anything in theatre and anything in the emergency trolley.

Asset spot check. Ten significant items from the register, found and working. Then ten significant items seen in the building, found on the register.

Cash. Today's takings so far, against the till or the record, against the receipt book.

Referrer harvest, and this one is a deliverable rather than a check. Osteon has no referrer list, so we build it on the day. Work back through the registration forms and first-clinic notes for the last 24 months and pull out every named referring doctor, physiotherapist or clinic, with the date and the patient count. A practice almost always records "referred by" on the front sheet even when nobody keeps a register, so the



list exists as scattered ink rather than as a document. Add to it the names patients give us in the patient survey, and the names each referrer nominates in theirs. Deduplicate, then segment by whether the last referral was recent, fading or over a year old, because the doctor who stopped is the most valuable conversation available and he is invisible without this exercise. Photograph or transcribe on site; nothing identifiable about the patient leaves with us, only the referrer's name.

Interview guides

Thirty to forty minutes each. Open with the standard opener. Let silences run.

Dr Bola, and this one needs real time

- Walk me through a good week and a bad week, and tell me what makes the difference between them.
- Where do you personally think the business loses money, and what have you already tried.
- Of the cases you did at Cedarcrest and Diamed in the last year, how many could have been done here, and what stopped each one.
- When you are operating elsewhere, who is in charge here, and what are they allowed to decide without ringing you.
- What happens here if a patient deteriorates at four in the morning. Talk me through it.
- If a manufacturer recalled an implant batch tomorrow, how would you find the patients.
- If we published your revision and infection rates next month, what would they say, and are you comfortable with that.
- What is the one thing you would fix tomorrow if you could fix only one.
- What would make you comfortable letting another surgeon operate under the Osteon name.
- What do you want this to be in five years, honestly, including the version where it stays personal.

Whoever runs the clinic day to day

- Walk me through a normal day, start to finish, and tell me where it breaks.
- What do you decide yourself, and what has to wait for Dr Bola.
- What does a busy day look like, and what gets dropped on one.
- How does a patient get from ringing the phone to sitting in front of the surgeon.
- Where do enquiries go when nobody answers, and who checks them.
- What do you not have that would make the biggest difference.

Theatre and nursing lead

- Talk me through a joint replacement list from the moment the first patient is sent for.
- What time is the list meant to start, and what time does it actually start, and why.
- What is the longest gap between two cases in a normal list, and what is happening in it.
- How often is a case cancelled on the day, and what causes it.
- Who sterilises the instruments, and how do you know a set is sterile when you open it.
- Where does the implant sticker go, and who is responsible for it.
- When a loan kit arrives late or short, what happens.
- What routines are meant to happen every single list, and which ones survive a busy day.
- Tell me about a time something went wrong. What happened afterwards.

Anaesthetist, including a visiting one if possible



- How are you booked, how are you paid, and what happens if you cannot come.
- What pre-operative information do you get, and when.
- What would make you refuse to proceed with a case here.
- What is the plan if a patient needs intensive care from this theatre.
- What monitoring and drugs do you have, and what do you bring yourself.
- How does recovery work here, and who is watching the patient.

Front desk and patient coordinator

- Walk me through a patient from the first phone call to the day of surgery.
- How many people ring in a week, and where is that written down.
- What do you say when somebody asks the price.
- What is the most common reason a patient who wanted surgery does not have it.
- What happens when a patient says they cannot afford it.
- Who handles a family abroad who wants to pay, and how does that work.

Accounts, billing and claims

- How is an invoice produced, and by whom.
- How do you know what a case should cost before it is billed.
- How is a claim submitted, chased and reconciled, and what is the oldest one outstanding.
- What do you not trust in the current numbers, and why.
- How do you know at the end of a month what the clinic earned.

Physiotherapist

- What proportion of post-operative patients do you actually see, and for how long.
- Is there a protocol, and does anyone follow it.
- What happens to a patient who lives far away.
- What would you change about how patients are prepared before surgery.

Any staff member, informally, whenever there is a quiet moment

- How long have you been here, and what keeps you here.
- If you could change one thing, what would it be.
- Who do you go to when something is wrong.

Live observation

Time a clinic consultation. With the patient's permission, sit in on one full consultation. Record: waiting time before it started, duration, whether imaging was available at the start, whether a price was discussed, whether the patient left knowing what happens next and who they call.

Time a theatre list. Send-for time, patient into anaesthetic room, knife to skin, closure, out of theatre, next patient in. Count door openings and people in the room. Note who scrubs, who breaks scrub, and what for.

Watch the front desk for one hour. Count calls, count people, note how many enquiries end with a name and number written somewhere retrievable.

Watch a handover, if one happens. Structured or not, what is lost.



What we must not leave without

- The ten case numbers and their twelve numbers each
- Photographs of the autoclave plate, one week of cycle logs, the emergency trolley, the implant store, the theatre, and the sterilisation flow
- The implant stock count sheet
- A written list, handed to the point of contact before we go, of every document still outstanding with a date against each
- The harvested referrer list, deduplicated, with last-referral dates
- Agreement on when we come back for a theatre list if we did not see one

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